



Bill Number: S.B. 1713

Shope Floor Amendment

**Reference to: HEALTH & HUMAN SERVICES
Committee Amendment**

Amendment drafted by: Leg. Council

FLOOR AMENDMENT EXPLANATION

1. Delays, from October 1, 2026, to October 1, 2027, the deadline for the Arizona Health Care Cost Containment System to implement the Arizona hybrid procurement model.
2. Makes technical changes.

SHOPE FLOOR AMENDMENT
SENATE AMENDMENTS TO S.B. 1713
(Reference to HEALTH AND HUMAN SERVICES Committee amendment)

Amendment instruction key:

[GREEN UPPERCASE UNDERLINING IN BRACKETS] indicates that the amendment is adding text to statute or previously enacted session law.
[Green lowercase underlining in brackets] indicates that the amendment is adding text to new session law or is restoring previously stricken text to existing statute.
~~[GREEN UPPERCASE STRIKEOUT IN BRACKETS]~~ indicates that the amendment is removing new text from statute or previously enacted session law.
~~[Green lowercase strikeout in brackets]~~ indicates that the amendment is removing text from existing statute, previously enacted session law or new session law.
<<Double green carets enclosing an entire section>> indicates that the amendment is adding the section to the bill.
~~<<Green strikeout with double green carets enclosing an entire section>>~~ indicates that the amendment is removing the section to the bill.
{[ORANGE UPPERCASE UNDERLINING IN DOUBLE CURLY BRACKETS]} indicates that the amendment to an amendment is adding text to statute or previously enacted session law.
{[Orange lowercase underlining in double curly brackets]} indicates that the amendment to an amendment is adding text to new session law or is restoring previously stricken text to existing statute.
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>>Double orange underlined carets enclosing an entire section>> indicate that the amendment to an amendment is adding the section to the bill.
~~>>Orange strikeout with double orange underlined carets enclosing an entire section>>~~ indicates that the amendment to an amendment is removing the section from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Title 36, chapter 29, Arizona Revised Statutes, is
3 amended by adding article 3.1, to read:

4 ARTICLE 3.1. PROCUREMENT AND CONTRACTING

5 36-2978. Definitions

6 IN THIS ARTICLE, UNLESS THE CONTEXT OTHERWISE REQUIRES:

7 1. "ADMINISTRATION" MEANS THE ARIZONA HEALTH CARE COST CONTAINMENT
8 SYSTEM ADMINISTRATION.

9 2. "ARIZONA HYBRID MODEL" MEANS A PERFORMANCE-BASED PROCUREMENT AND
10 CONTRACT LIFE CYCLE FRAMEWORK THAT EVALUATES CONTRACTORS BASED ON
11 OBJECTIVE, MEASURABLE OPERATIONAL PERFORMANCE OVER A DEFINED MULTI-YEAR
12 PERIOD, SUBJECT TO THRESHOLD OPERATIONAL READINESS REQUIREMENTS.

13 3. "CONTRACTING DETERMINATION" MEANS A DETERMINATION BY THE
14 ADMINISTRATION REGARDING A CONTRACTOR'S ELIGIBILITY, SELECTION,
15 CONTINUATION, EXPANSION, LIMITATION, SUSPENSION OR TERMINATION UNDER THIS
16 ARTICLE.

17 4. "CONTRACTOR":

1 (a) HAS THE SAME MEANING PRESCRIBED IN SECTION 36-2901.

2 (b) INCLUDES ENTITIES COMMONLY REFERRED TO AS MANAGED CARE
3 ORGANIZATIONS.

4 5. "INCUMBENT CONTRACTOR" MEANS A CONTRACTOR THAT IS SERVICING A
5 PROGRAM IN A GENERAL SERVICE AREA AS OF 2026.

6 6. "NONINCUMBENT CONTRACTOR" MEANS AN EXISTING OR POTENTIAL
7 CONTRACTOR THAT WISHES TO SERVICE A PROGRAM THAT THE CONTRACTOR DOES NOT
8 CURRENTLY SERVICE IN A GENERAL SERVICE AREA PURSUANT TO THIS ARTICLE.

9 7. "OPERATIONAL FACTORS" MEANS ELEMENTS RELATED TO THE
10 ADMINISTRATION OF BENEFITS AND ACCESS TO CARE AND SERVICES THAT ARE
11 SUBJECT TO EVALUATION UNDER THE PERFORMANCE RUBRIC USING QUANTITATIVE
12 STANDARDS ESTABLISHED BY THE ADMINISTRATION, INCLUDING CLAIMS PROCESSING
13 AND PAYMENT, APPEALS, NETWORK ADEQUACY AND ENCOUNTER DATA SUBMISSION.

14 8. "OPERATIONAL READINESS" MEANS A THRESHOLD DETERMINATION THAT A
15 CONTRACTOR MEETS MINIMUM OPERATIONAL, ADMINISTRATIVE, FINANCIAL AND
16 SYSTEMS REQUIREMENTS NECESSARY TO PERFORM UNDER A MANAGED CARE CONTRACT,
17 AS ESTABLISHED BY THE ADMINISTRATION BY RULE.

18 9. "PERFORMANCE AND OPERATIONS PHASE" MEANS A DEFINED INITIAL
19 PERIOD OF OPERATIONS LASTING FOUR YEARS DURING WHICH CONTRACTOR
20 PERFORMANCE IS EVALUATED PURSUANT TO THE PERFORMANCE RUBRIC FOR THE
21 PURPOSES OF CONTRACT CONTINUATION, EXPANSION, LIMITATION, SUSPENSION OR
22 TERMINATION.

23 10. "PERFORMANCE RUBRIC" MEANS A STANDARDIZED SET OF QUANTITATIVE
24 MEASURES THAT IS USED TO:

25 (a) EVALUATE CONTRACTOR PERFORMANCE, INCLUDING RATES, RATIOS,
26 TIMELINESS BENCHMARKS AND ACCURACY THRESHOLDS, AND ADDRESS QUALITY OF
27 CARE, MEMBER EXPERIENCE AND OPERATIONAL FACTORS RELATED TO THE
28 ADMINISTRATION OF BENEFITS AND ACCESS TO CARE AND SERVICES.

29 (b) MEASURE PERFORMANCE OVER THE LIFE OF A CONTRACT AND THAT IS THE
30 ONLY MEANS OF ASSESSING COST, QUALITY AND MEMBER AND PROVIDER SATISFACTION
31 FOR THE PURPOSES OF CONTRACT AWARD, AMENDMENT OR RECISSION.

32 11. "PROGRAM" MEANS ANY OF THE FOLLOWING:

33 (a) THE ARIZONA HEALTH CARE COST CONTAINMENT SYSTEM PURSUANT TO
34 ARTICLE 1 OF THIS CHAPTER.

35 (b) THE ARIZONA LONG-TERM CARE SYSTEM, AS IT APPLIES TO PERSONS WHO
36 ARE ELDERLY OR WHO HAVE A PHYSICAL DISABILITY PURSUANT TO ARTICLE 2 OF
37 THIS CHAPTER.

38 (c) THE CHILDREN'S HEALTH INSURANCE PROGRAM PURSUANT TO ARTICLE 4
39 OF THIS CHAPTER.

40 36-2978.01. Managed care contracts; procurement

41 NOTWITHSTANDING TITLE 41, CHAPTERS 6 AND 23 OR ANY OTHER LAW TO THE
42 CONTRARY:

43 1. BEGINNING OCTOBER 1, ~~{{2026}}~~ {{2027}}, THE ADMINISTRATION SHALL
44 CONDUCT MANAGED CARE CONTRACTING FOR ANY PROGRAM EXCLUSIVELY PURSUANT TO
45 THIS ARTICLE. THIS ARTICLE APPLIES ONLY TO COMPREHENSIVE, RISK-BASED
46 MANAGED CARE CONTRACTS ISSUED BY THE ADMINISTRATION. THE ADMINISTRATION
47 SHALL NOT ISSUE REQUESTS FOR PROPOSALS, COMPETITIVE SEALED PROPOSALS OR
48 SIMILAR SOLICITATIONS FOR MANAGED CARE CONTRACTORS.

1 2. ALL SELECTION, EVALUATION, CONTINUATION, EXPANSION, LIMITATION,
2 SUSPENSION AND TERMINATION OF CONTRACTORS SHALL BE CONDUCTED SOLELY
3 PURSUANT TO THIS ARTICLE, WHICH CONSTITUTES THE EXCLUSIVE MANAGED CARE
4 CONTRACTING FRAMEWORK FOR EACH PROGRAM.

5 36-2978.02. Arizona hybrid model; performance-based
6 procurement framework; implementation

7 A. THE ARIZONA HYBRID MODEL SHALL INCLUDE ONLY THE FOLLOWING:

8 1. A PERFORMANCE-BASED SELECTION AND RETENTION OF CONTRACTORS USING
9 DEMONSTRATED PERFORMANCE AGAINST A PERFORMANCE RUBRIC AND THE OPERATIONAL
10 FACTORS.

11 2. AN EVALUATION OF PERFORMANCE OVER AN INITIAL DEFINED PERFORMANCE
12 AND OPERATIONS PHASE LASTING FOUR YEARS. THE RESULTS OF THE PERFORMANCE
13 AND OPERATIONS PHASE CONSTITUTE THE PRIMARY AND CONTROLLING BASIS FOR THE
14 ADMINISTRATION'S DECISIONS REGARDING CONTRACT CONTINUATION, EXPANSION,
15 LIMITATION, SUSPENSION AND TERMINATION. AFTER COMPLETING THE INITIAL
16 PERFORMANCE AND OPERATIONS PHASE, CONTRACTORS REMAIN SUBJECT TO ONGOING
17 PERFORMANCE REVIEW, MONITORING AND ENFORCEMENT BASED ON THE PERFORMANCE
18 RUBRIC AND OPERATIONAL FACTORS.

19 3. EVALUATION STANDARDS THAT ARE OBJECTIVE, MEASURABLE, PUBLISHED
20 IN ADVANCE AND APPLIED UNIFORMLY AND THAT DO NOT INCLUDE SUBJECTIVE,
21 NARRATIVE-BASED OR DISCRETIONARY EVALUATION CRITERIA.

22 4. THE ESTABLISHMENT AND ENFORCEMENT OF MINIMUM ENROLLMENT
23 VIABILITY STANDARDS AND USE OF A SINGLE MEDICAID MANAGED CARE CONTRACT
24 FRAMEWORK FOR ALL CONTRACTORS THAT SERVICE A PROGRAM, INCLUDING UNIFORM
25 PERFORMANCE EXPECTATIONS, EVALUATION STANDARDS AND RENEWAL MECHANISMS TO
26 ELIMINATE DUPLICATIVE PROCUREMENTS AND ASSOCIATED ADMINISTRATIVE BURDEN.

27 B. NOTWITHSTANDING TITLE 41, CHAPTERS 6 AND 23 OR ANY OTHER LAW TO
28 THE CONTRARY, THE ADMINISTRATION SHALL USE A MULTI-PHASE PROCUREMENT AND
29 PERFORMANCE STRUCTURE TO IMPLEMENT THE ARIZONA HYBRID MODEL, CONSISTING OF
30 ALL OF THE FOLLOWING:

31 1. ON OR BEFORE OCTOBER 1, ~~{{2026}}~~ {{2027}}, THE ADMINISTRATION
32 SHALL DO BOTH OF THE FOLLOWING AND MAKE THE INFORMATION PUBLICLY
33 AVAILABLE:

34 (a) DEVELOP A STANDARDIZED PERFORMANCE RUBRIC. ALL METHODOLOGIES,
35 MEASURES, BENCHMARKS AND CALCULATION METHODS USED IN THE PERFORMANCE
36 RUBRIC SHALL BE PUBLICLY AVAILABLE THROUGHOUT THE DEVELOPMENT PROCESS, BUT
37 CONTRACTOR-SPECIFIC UNDERLYING DATA, PROPRIETARY INFORMATION AND PROTECTED
38 HEALTH INFORMATION MAY NOT BE DISCLOSED. THE ADMINISTRATION SHALL
39 DEVELOP, PUBLISH AND INVITE FEEDBACK ON THE PERFORMANCE RUBRIC BEFORE
40 COMMENCING THE CONTRACTING ELIGIBILITY DETERMINATION PERIOD REQUIRED BY
41 THIS ARTICLE.

42 (b) ADOPT RULES, CONSISTENT WITH FEDERAL LAW, PROVIDING FOR A
43 PERFORMANCE BOND REQUIREMENT FOR POTENTIAL CONTRACTORS OR CONTRACTORS
44 SEEKING TO PARTICIPATE. EACH POTENTIAL CONTRACTOR OR CONTRACTOR SHALL
45 ATTEST, BEFORE BOTH THE CONTRACTING ELIGIBILITY DETERMINATION PERIOD AND
46 THE PERFORMANCE AND OPERATIONS PHASE, THAT THE POTENTIAL CONTRACTOR OR
47 CONTRACTOR UNDERSTANDS THE PARAMETERS AND REQUIREMENTS OF THE PERFORMANCE
48 RUBRIC, ACCEPTS THE STATISTICAL AND METHODOLOGICAL BASIS ON WHICH THE
49 PERFORMANCE RUBRIC IS CONSTRUCTED AND AGREES TO ABIDE BY THE APPLICATION

1 OF THE PERFORMANCE RUBRIC PURSUANT TO SECTION 36-2978.05. EACH POTENTIAL
2 CONTRACTOR OR CONTRACTOR THAT DOES NOT MEET THE ESTABLISHED STANDARDS MAY
3 BE REMOVED AND IS NOT ELIGIBLE TO CONTRACT FOR A PROGRAM IN ACCORDANCE
4 WITH THIS ARTICLE AND THE RULES ADOPTED PURSUANT TO THIS ARTICLE.

5 2. TO STREAMLINE ADMINISTRATIVE PROCESSES, THE ADMINISTRATION SHALL
6 USE A SINGLE STATEWIDE MASTER CONTRACT FOR CONTRACTORS WITH
7 PROGRAM-SPECIFIC SCHEDULES, EXHIBITS OR ATTACHMENTS INCLUDED AS NECESSARY
8 TO SATISFY APPLICABLE FEDERAL REQUIREMENTS.

9 3. THE ADMINISTRATION SHALL ESTABLISH A BALANCED, TRANSPARENT
10 ENROLLMENT STRUCTURE ACROSS ALL INCUMBENT CONTRACTORS AND NONINCUMBENT
11 CONTRACTORS THAT ADVANCES MARKET FAIRNESS, SUPPORTS ONGOING STATEWIDE
12 COMPETITION AND PROTECTS MEMBER CHOICE WHILE MAINTAINING COMPLIANCE WITH
13 APPLICABLE FEDERAL REGULATIONS. THE ADMINISTRATION SHALL ADOPT RULES THAT
14 INCLUDE THE FOLLOWING:

15 (a) PROCEDURES AUTHORIZING THE ADMINISTRATION TO DIRECT, ON A
16 TEMPORARY BASIS, ALL OR A SUBSTANTIAL PORTION OF AUTO-ASSIGNED ENROLLMENT
17 TO NONINCUMBENT CONTRACTORS UNTIL THE ADMINISTRATION DETERMINES THAT THE
18 MINIMUM ENROLLMENT LEVELS NECESSARY FOR OPERATIONAL VIABILITY HAVE BEEN
19 ACHIEVED AND THAT THE MEMBERS RECEIVE NOTICE AND RETAIN THE RIGHT TO
20 SELECT ANOTHER AVAILABLE CONTRACTOR.

21 (b) PROCEDURES FOR THE BALANCED AND TRANSPARENT CHANGE OF
22 ENROLLMENT OF CONTRACTORS AS A RESULT OF THE OUTCOME OF THE PERFORMANCE
23 AND OPERATIONS PHASE.

24 (c) PROCEDURES REQUIRING THE ADMINISTRATION TO ANNUALLY DECIDE
25 WHETHER SMALLER OR LARGER CONTRACTORS HAVE DIFFERENT PER MEMBER
26 ADMINISTRATIVE OR OPERATING COSTS, TO PUBLISH THAT DECISION AS PART OF
27 RATE SETTING AND TO APPLY THAT RATIONALE WHEN SUPPORTED BY ACTUARIAL
28 ANALYSIS OR EXPLAIN WHY THE DECISION WAS NOT APPLIED.

29 (d) PROCEDURES FOR ANNUALLY UPDATING ENROLLMENT AFTER THE
30 PERFORMANCE AND OPERATIONS PHASE FOR ALL CONTRACTORS BASED ON HIGH AND LOW
31 PERFORMANCE, INCLUDING WHETHER IT IS NECESSARY TO MAKE SUCH ADJUSTMENTS.

32 (e) PROCEDURES GOVERNING THE ORDERLY ENROLLMENT TRANSITION FOR
33 MEMBERS OF CONTRACTORS THAT FAIL TO MEET ESTABLISHED PERFORMANCE
34 STANDARDS, IN A MANNER THAT MINIMIZES DISRUPTION FOR MEMBERS OF THE
35 PROGRAMS.

36 (f) PROCEDURES TO ENSURE THAT A CONTRACTOR DOES NOT RECEIVE OR IS
37 NOT AUTO-ASSIGNED MEMBERS IN A MANNER THAT RESULTS IN THE CONTRACTOR
38 EXCEEDING TWENTY PERCENT OF TOTAL ENROLLMENT IN ANY PROGRAM IN ANY YEAR.
39 THIS SUBDIVISION DOES NOT LIMIT VOLUNTARY MEMBER CHOICE. WHEN PRESENTING
40 CONTRACTOR OPTIONS FOR MEMBER SELECTION OR CONDUCTING AUTO-ENROLLMENT, THE
41 ADMINISTRATION SHALL LIMIT SUCH OPTIONS TO CONTRACTORS THAT ARE BELOW THE
42 TWENTY PERCENT ENROLLMENT THRESHOLD.

43 4. THE ADMINISTRATION SHALL CONDUCT A SINGLE, COORDINATED
44 PROCUREMENT USING THE METHOD REQUIRED BY THIS ARTICLE FOR CAPITATED,
45 COMPREHENSIVE MANAGED CARE ORGANIZATION CONTRACTS ISSUED BY THE
46 ADMINISTRATION, TO THE GREATEST EXTENT PRACTICABLE AND CONSISTENT WITH
47 FEDERAL LAW.

1 36-2978.03. Multiphase procurement; contractors; eligibility
2 to contract

3 A. THE ADMINISTRATION SHALL ESTABLISH A ~~{{MULTI-PHASE}}~~
4 ~~{{MULTIPHASE}}~~ PROCUREMENT THAT INCLUDES:

- 5 1. THE CONTRACTING ELIGIBILITY DETERMINATION PERIOD.
- 6 2. THE OPERATIONAL READINESS REVIEW.
- 7 3. THE PERFORMANCE AND OPERATIONS PHASE.
- 8 4. THE OPERATIONAL CAP AND MARKET ADJUSTMENT.

9 B. EACH INCUMBENT CONTRACTOR MAY FILE FOR ELIGIBILITY TO COMPETE
10 FOR A CONTRACT TO SERVE A PROGRAM OR A SERVICE AREA THAT IS NOT CURRENTLY
11 SERVED BY A MANAGED CARE ORGANIZATION. AN INCUMBENT CONTRACTOR CURRENTLY
12 SERVING ANY PROGRAM FOR THE ADMINISTRATION IS PRESUMED TO BE ELIGIBLE TO
13 PARTICIPATE IN THE PERFORMANCE AND OPERATIONS PHASE.

14 C. EACH NONINCUMBENT CONTRACTOR SHALL MEET BASIC ELIGIBILITY
15 CRITERIA PRESCRIBED BY THE ADMINISTRATION, INCLUDING FINANCIAL SOLVENCY,
16 AT LEAST FIVE YEARS OF OPERATIONAL EXPERIENCE, WHICH MAY BE IN OTHER
17 STATES, AND SUCCESSFUL OPERATIONAL READINESS HISTORY.

18 D. ANY APPLICANT THAT MEETS THE THRESHOLD ELIGIBILITY REQUIREMENTS
19 BUT THAT IS NOT INITIALLY SELECTED MAY BE PLACED IN A QUALIFIED APPLICANT
20 HOLDING POOL. THE ADMINISTRATION MAY ESTABLISH A CAP ON THE NUMBER OF
21 APPLICANTS THAT MAY FILE FOR A CONTRACTING ELIGIBILITY DETERMINATION.

22 36-2978.04. Applicants; operational readiness review;
23 notification; cap

24 A. ONLY APPLICANTS THAT ARE DETERMINED TO BE OPERATIONALLY READY
25 MAY ADVANCE BEYOND THE OPERATIONAL READINESS REVIEW PHASE. THE
26 ADMINISTRATION MAY REMOVE FROM CONSIDERATION ANY APPLICANT THAT IS
27 DETERMINED TO NOT BE OPERATIONALLY READY. THE REMOVAL OF AN APPLICANT
28 DOES NOT CONSTITUTE A CONTRACTING DETERMINATION AND IS NOT SUBJECT TO
29 INDEPENDENT REVIEW OR APPEAL. THE ADMINISTRATION SHALL NOTIFY THE
30 APPLICANT OF THE REASONS FOR THE REMOVAL.

31 B. THE ADMINISTRATION MAY ESTABLISH A CAP ON THE NUMBER OF
32 APPLICANTS THE ADMINISTRATION APPROVES AFTER THE OPERATIONAL READINESS
33 REVIEW.

34 36-2978.05. Performance period; operational caps; office of
35 administrative hearings review

36 A. CONTRACTS ISSUED PURSUANT TO THIS ARTICLE SHALL OPERATE FOR A
37 FOUR-YEAR PERFORMANCE PERIOD CONSISTENT WITH THIS SECTION. A CONTRACTOR'S
38 PERFORMANCE SHALL BE MEASURED EXCLUSIVELY USING THE PERFORMANCE RUBRIC AND
39 PUBLISHED OPERATIONAL FACTORS. THE ADMINISTRATION'S PERFORMANCE
40 DETERMINATIONS SHALL BE MATHEMATICAL, OBJECTIVE AND NONDISCRETIONARY.

41 B. THE ADMINISTRATION MAY IMPOSE CAPS ON THE TOTAL NUMBER OF
42 CONTRACTORS IN EACH PROGRAM. THE ADMINISTRATION'S DECISIONS REGARDING THE
43 CONTINUATION, EXPANSION, LIMITATION AND TERMINATION OF A CONTRACT SHALL BE
44 BASED ON PERFORMANCE OUTCOMES AND COHORT LIMITATIONS.

45 C. IF A CONTRACTOR FILES A PROTEST OR APPEAL REGARDING THE FINAL
46 APPLICATION OF THE PERFORMANCE RUBRIC PURSUANT TO THIS SECTION AND HAS
47 EXHAUSTED THE APPLICABLE PROTEST OR APPEAL PROCESS, THE OFFICE OF
48 ADMINISTRATIVE HEARINGS IS LIMITED TO VERIFYING WHETHER THE ADMINISTRATION
49 APPLIED ITS PUBLISHED PERFORMANCE RUBRIC CONSISTENTLY AND OBJECTIVELY,

1 MAINTAINED DATA INTEGRITY, INCLUDING THE USE OF CORRECT SOURCES AND TIME
2 FRAMES, AND APPLIED FORMULAS, WEIGHTS AND CALCULATIONS EXACTLY AS
3 PUBLISHED. IF THE OFFICE OF ADMINISTRATIVE HEARINGS CONFIRMS AN ERROR,
4 THE ADMINISTRATION SHALL CORRECT AND REISSUE THE DETERMINATION. THE
5 DECISION OF THE OFFICE OF ADMINISTRATIVE HEARINGS IS FINAL AND NO FURTHER
6 ADMINISTRATIVE APPEAL IS AVAILABLE.

7 ~~[36-2978.06. Hospital contracting standards~~

8 ~~A. AS A CONDITION OF CONTRACTING UNDER THIS ARTICLE, CONTRACTORS~~
9 ~~AND ANY SUBCONTRACTORS SHALL ENSURE THAT HOSPITAL CONTRACTING PRACTICES DO~~
10 ~~NOT RESTRICT COMPETITION OR ACCESS TO CARE. A HOSPITAL THAT IS OWNED,~~
11 ~~OPERATED BY OR AFFILIATED WITH A CONTRACTOR OR SUBCONTRACTOR MAY NOT~~
12 ~~REFUSE TO CONTRACT WITH ANY OTHER CONTRACTOR OR SUBCONTRACTOR THAT IS~~
13 ~~AUTHORIZED TO OPERATE IN THE HOSPITAL'S SERVICE AREA.~~

14 ~~B. CONTRACTS BETWEEN CONTRACTORS AND HOSPITALS SHALL BE NEGOTIATED~~
15 ~~IN GOOD FAITH AND ON REASONABLE AND NONDISCRIMINATORY TERMS. A HOSPITAL~~
16 ~~MAY NOT USE EXCLUSIVITY PROVISIONS OR ENROLLMENT-BASED OR VOLUME-BASED~~
17 ~~RESTRICTIONS AS REASONS TO NOT CONTRACT WITH A CONTRACTOR.]~~

18 ~~[36-2978.07.][36-2978.06.]~~ Federal compliance; program
19 protection; rulemaking

20 A. A CONTRACT MAY NOT BE AWARDED, CONTINUED OR RETAINED IF DOING SO
21 WOULD RESULT IN THE LOSS OF FEDERAL MEDICAID FUNDING.

22 B. THIS ARTICLE DOES NOT ALTER THE ADMINISTRATION'S AUTHORITY OR
23 OBLIGATION TO COMPLY WITH FEDERAL MEDICAID REQUIREMENTS OR APPLICABLE
24 WAIVER AUTHORITIES.

25 C. THIS ARTICLE DOES NOT CHANGE ELIGIBILITY CATEGORIES, MEMBER
26 POPULATIONS, BENEFIT DESIGN OR SERVICE DELIVERY MODELS, INCLUDING
27 STATE-ADMINISTERED OR FEE-FOR-SERVICE PROGRAMS.

28 D. THE ADMINISTRATION SHALL ADOPT RULES NECESSARY TO IMPLEMENT THIS
29 ARTICLE CONSISTENT WITH ITS REQUIREMENTS.

30 Sec. 2. Rulemaking exemption; public hearing

31 Notwithstanding any other law, for the purposes of this act, the
32 Arizona health care cost containment system is exempt from the rulemaking
33 requirements of title 41, chapters 6 and 6.1, Arizona Revised Statutes,
34 except that the Arizona health care cost containment system shall provide
35 notice and at least one public hearing at least thirty days before
36 finalizing the rules.

37 Sec. 3. Legislative findings; intent

38 A. The legislature finds that the Arizona health care cost
39 containment system should operate as a national leader in the
40 administration of medicaid managed care contracting and that competition
41 is best achieved through demonstrated performance rather than speculative
42 or proposal-based projections.

43 B. The legislature further finds that procurement models relying
44 primarily on subjective or narrative evaluation criteria increase
45 litigation risk, delay implementation of contracts and create uncertainty
46 for providers, members and this state.

1 C. The legislature determines that a performance-based procurement
2 and contract life cycle framework using objective, measurable metrics
3 applied over time promotes transparency, accountability, market stability
4 and fiscal stewardship.

5 D. The legislature intends to require the Arizona health care cost
6 containment system to implement a mandatory and uniform managed care
7 procurement and contracting framework that selects and retains contractors
8 based on demonstrated performance rather than hypothetical or
9 narrative-based submissions.

10 Enroll and engross to conform

11 Amend title to conform

THOMAS "T.J." SHOPE

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02/25/2026

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